INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Docetaxel 100 mg/m² · every 3 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00202
Version 8

> Docetaxel 100 mg/m² by intravenous (IV) infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

Treatment stages

01 **Docetaxel**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Docetaxel 100 mg/m² · every 3 weeks
National Cancer Control Programme (NCCP) 00202

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Control advanced cancer and symptoms while balancing side effects and treatment burden.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell's division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

Docetaxel is a chemotherapy option for locally advanced or metastatic breast cancer. Previous anthracyclines or taxanes, neuropathy, liver function, fitness and available endocrine or targeted options affect selection. The 100 mg/m² schedule has its own tolerability and dose-reduction rules.

Your treatment setting

Locally advanced or metastatic breast cancer

Your exact schedule

Docetaxel 100 mg/m² by intravenous (IV) infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Docetaxel 100 mg/m² · every 3 weeks
NCCP 00202

Treatment and review

Docetaxel

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY INTERVAL

Docetaxel

Repeated cycles

Docetaxel 100 mg/m² by intravenous (IV) infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00202 v8 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel 100 mg/m² · every 3 weeks: treatment and checks

Locally advanced or metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00202	Baseline eligibility / tests	Day 1 infusion	Repeat every 21 days	Until progression / intolerance	Next treatment plan
Blood / organ function NCCP 00202	full blood count (FBC), renal, liver baseline	Review results before dose	FBC / renal / liver every cycle	Review recovery before restart	Investigate persisting problems

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Steroids / symptoms NCCP 00202	Dexamethasone from day before	Confirm ≥3 pre-treatment doses	Neuropathy, fluid retention, infection	Review reductions / recurrence	Report persistent symptoms
Cancer review METASTATIC · MARKERS	Confirm setting	Agree review plan	Response / symptom review	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Locally advanced or metastatic breast cancer. FBC, kidney and liver tests before treatment and every cycle. Review infection, neuropathy, skin/nail toxicity and swelling before the next dose.

Ask / act: Confirm eligibility, test results and medicine list.

2 Other checks

At the specified review points

Review premedication adherence and cumulative fluid retention. Cardiac investigations are based on symptoms and individual risk, not a routine HER2 schedule.

Ask / act: Confirm who arranges and reviews each check.

3 Support and practical plan

At each treatment or phase change

Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any IV replacement is a consultant decision.

Ask / act: Choose the matching support plan; confirm active medicines.

4 Response and follow-up

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your treatment and access plan

STEP 1

Before treatment

FBC, kidney and liver tests before treatment and every cycle. Review infection, neuropathy, skin/nail toxicity and swelling before the next dose.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Docetaxel infusion is about one hour. Blood review, preparation, premedication and observation add to the visit.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any IV replacement is a consultant decision.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

Other treatment-specific checks

Review premedication adherence and cumulative fluid retention. Cardiac investigations are based on symptoms and individual risk, not a routine HER2 schedule.

Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Docetaxel

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Docetaxel

Treatment or check	Timing in this view
Chemotherapy · Day 1	 **Day 1** IV
Administration reaction	 **Days 1–21** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–21** Variable timing and severity
Nerves / skin / nails	 **Days 1–21** Can build across doses

Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00202 v8 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–21: Reference cycle

Docetaxel 100 mg/m² by IV infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

Support around the medicines

1 **Blood counts and infection**

Fever, shivers or sudden illness needs prompt advice on any day.

2 **Nerve symptoms**

Report numbness, tingling or weakness before the next dose.

3 **Support medicines**

Follow the prescription for the active medicine and treatment day.

4 **Review the plan**

Discuss symptoms, recovery and the next treatment decision with the team.

Questions worth asking

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Try explaining the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Taxane / combination phase

Active medicines: Docetaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any intravenous (IV) replacement is a consultant decision.
- Reconcile the total dexamethasone schedule with antiemetics, including aprepitant interactions. The listed antiemetic steroid is a reference row, not an instruction to add a second course.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to the exact regimen and individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Docetaxel 100 mg/m² · every 3 weeks

Docetaxel 100 mg/m² by intravenous (IV) infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

3 Hair / scalp

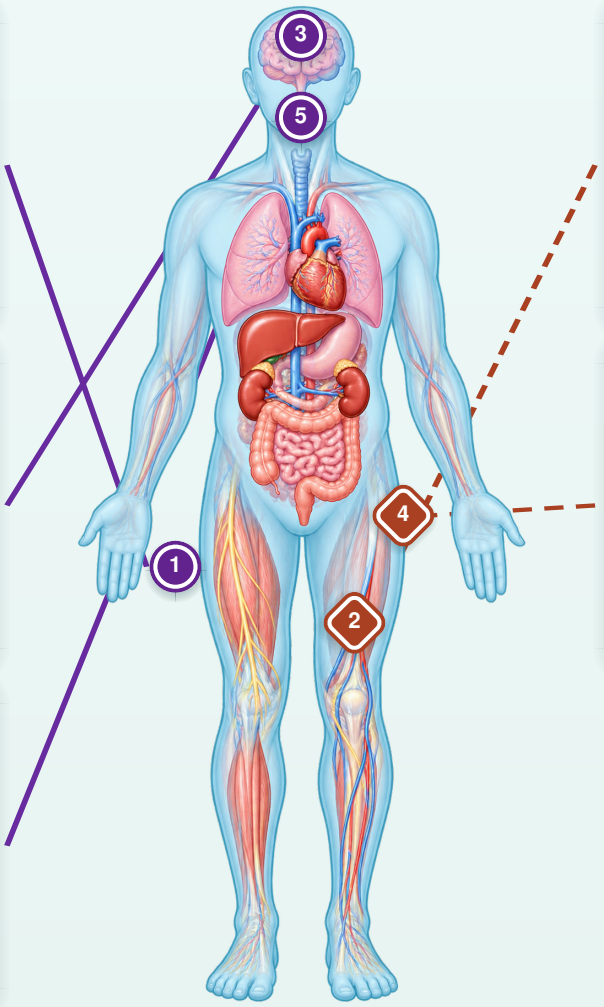
Docetaxel

Hair loss is common. Ask about scalp cooling before treatment where suitable.

5 Mouth

Docetaxel

Painful ulcers or difficulty drinking need advice.



2 Blood / infection

Docetaxel

Fever, shivers or suddenly unwell: call immediately.

4 Fluid / skin / nails

Docetaxel

Report swelling, painful skin or nail changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; this is not a complete list. Report new or persistent symptoms even after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Docetaxel · reusable record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Docetaxel 100 mg/m² · every 3 weeks

NCCP 00202 v8 · 21-day cycle

Docetaxel 100 mg/m² by IV infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

1 2 3 4 5 6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00202/ · no patient information.

Draft for review · SACTCheck v0.84.19

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00202 v8 · official regimen**
   https://healthservice.hse.ie/documents/6541/202_DOCEtaxel_100-21day.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf